

Reading the Skin

The consultation, common conditions, and knowing precisely where your practice ends

Learning Objectives

- Conduct a structured skin consultation and read the skin under proper light
- Identify and grade common conditions: acne, hyperpigmentation, sensitivity, and barrier damage
- Know precisely which presentations must be referred to a dermatologist, and why
- Begin your first real case — the client you will carry through this entire course

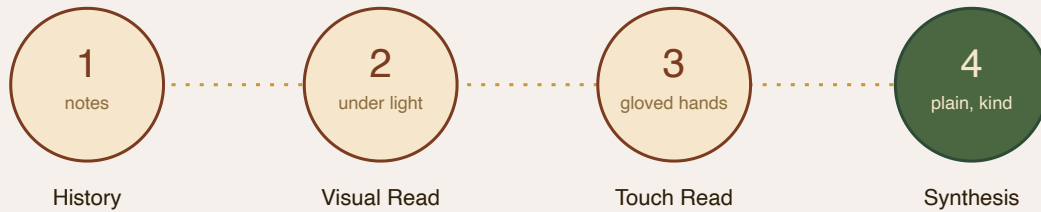
Before you treat, you must read

Every skilled practitioner shares one habit: they look, fully, before they act. In skin work, that looking has a name — the consultation — and it is not small talk before the real work begins. It *is* the real work beginning.

The structure of a consultation

1. **The history.** Current routine, known sensitivities, medications, lifestyle, and what genuinely troubles the client — in her own words.
2. **The visual read.** Under proper, ideally daylight-balanced light — never dim, flattering salon lighting. Note tone, texture, congestion, pigmentation, asymmetries.
3. **The touch read.** Clean, gloved hands assessing texture, oil, temperature, tenderness.
4. **The synthesis.** Naming, plainly and kindly, what you observe.

◆ Figure 3.1 · The Four-Step Consultation



The practitioner's opening ritual: history, visual read, touch read, synthesis.

Reading acne — grade before you treat

- **Grade I — mild.** Occasional blackheads and whiteheads. Fully within scope.
- **Grade II — moderate.** Frequent breakouts, some inflamed papules, often along the jawline. Within scope, with care.
- **Grade III — moderately severe.** Numerous inflamed lesions, early scarring risk. Treat conservatively; consider joint approach with a dermatologist.
- **Grade IV — severe (cystic).** Deep, painful nodules and cysts. **Not yours to extract or treat aggressively. Refer — always.**

Knowing what you must not touch is not a limitation of your skill. It is the proof of it.

Hyperpigmentation, sensitivity, and the compromised barrier

- **Hyperpigmentation** — including PIH — patches darker than surrounding tone, from acne, injury, sun, or hormonal change (melasma).
- **Sensitivity** — reactive, easily irritated skin, requiring the gentler philosophy of Module Six.
- **Barrier damage** — a compromised stratum corneum: tightness, dullness, flaking, or stinging at once-tolerated products.

The referral line

Refer to a dermatologist, always, for: cystic or nodular acne (Grade IV); any changing mole or lesion; suspected infection or unexplained bleeding; suspected fungal or bacterial conditions; or any presentation that unsettles your judgement. **When in doubt, refer.**

Your First Case · Meet Priya

Priya, 28, Fitzpatrick Type V, based in Mumbai. Grade II jawline acne, visible post-inflammatory hyperpigmentation from breakouts over the past year, skin tight and dull despite daily moisturiser, and a "fairness" cream of unknown actives she has used inconsistently for months.

Run Priya through the four-step consultation. Write your history notes, visual and touch observations, and synthesis. Do not prescribe yet — that comes once you have learned the ingredients (Module Four) and routines (Module Thirteen).

Carry Priya with you. You will complete her full skincare prescription by the end of this course.

Key Terms

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[object Object]

1

[object Object]

2

[object Object]

3

[object Object]

Practical Activity · Priya's Consultation Notes

Write Priya's complete consultation notes — history, visual read, touch read, synthesis. Grade her acne, name her hyperpigmentation, note the barrier concern beneath her dullness. **Save this — it is the opening page of the case study you submit at your capstone.**